



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متكرر

إسم المريض ربيعاً : هناء حسن علي حسن

المحافظة : الجيزة

الحي / المدينة : العمرانية

العنوان : 5ش ملطي سلامة -

عثمان محرم العمرانية - الجيزة

رقم البطاقة : 26502110103925 رقم التليفون : 01033032046

تاريخ تسجيل الإستمارة : 02-10-2026

العمر (تاريخ الميلاد) : 11-02-1965 المهنة : بدون عمل

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك الإخصائي الاجتماعي التوقيع: (-----) المقر بما فيه التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] قسم العلاج : الاورام

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني
الاسم : د/ندى حسن
الاسم : د/نرمين رمضان
الوظيفة :
الوظيفة :
الوظيفة :
التوقيع :
التوقيع :
التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة. تعتمد صحة البيانات المرفقة رئيس اللجنة ومدير المستشفى.
تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. / د. / د. / د. / د. / د. / د. / د.

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
قسم :
جهة العلاج :
نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

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Name: هناء حسن علي حسن National ID: 26502110103925	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

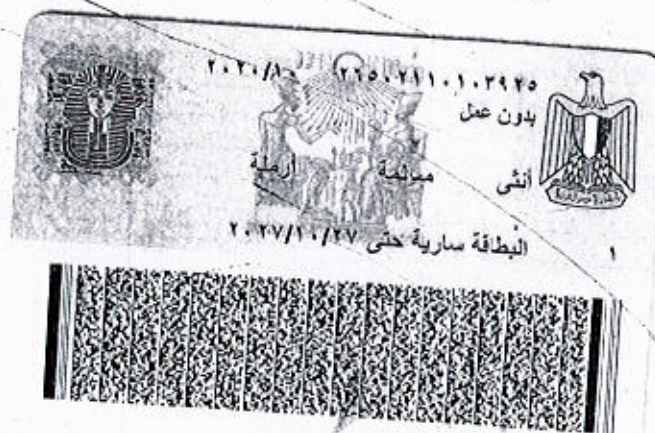
نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Supportive and palliative Treatment
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التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



MDT FORM

9/ 11/2025 G

Name	سيدة محمد حسين	MRN	01.07/0.01
Age	68y.	ID	000.01.00.00.00
Menopausal status	postmenop	Phone	01004926421
NRS SCORE		Marital status	
BMI		Family history	
CO morbidity	HTN	REASON OF MDT	

Clinical Data

BX: IDCT				R	PS3	Visceral	L
ER: tu 8/8	PR: tu 8/8	HER 2: -ve.	Ki67:				
Sonomammography: postmenop Metastatic Breast Cancer							
Mets Workup: History left Breast - Denovo HBC to Bone, Liver. St. femur 6/221 → 9/2024				Liver, Adrenal TNM			

Recommendations: PD of bilateral adrenal Met
BS 11/2024: PD
Rec. Faslodex, Ibrance, K3le for w/pow.
20.10.2024: PD of oscl. Stopped the sine 1/2025

MDT OUTCOME & PLAN CT, on 20/9/2024 PD of bilateral
Cpr of sgr vu in s.k.t.c.
✓ v/t. 20/9/24 PD of Adrenal Met T.BD1.3

Signature, (2) Zomera/3100

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
Dr. N. B. 6					

Name: هناة حسن علي حسن		
MR. : 3097		
Detailed Medical Report		
		السيد
تحتية طبية وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
ننشرف بأن نوضح فيما يلي تقريراً عن حالتها		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمتابعة وجد انها		
The patient is a 61-year-old individual who is post-menopausal and has a history of hypertension. The clinical history is notable for a rectovesical fistula with well-formed stool emanating from the urinary meatus. Metastatic workup demonstrated that colonoscopy was obstructed, with the scope unable to pass at 40 cm from the anal canal. PET-CT revealed a mixed solid and cystic soft-tissue mass inseparable from the uterine stump, urinary bladder roof, sigmoid, and rectosigmoid measuring 81 × 71 × 106 mm, left iliac region peritoneal deposits measuring 83 × 85 × 110 mm, hepatic focal lesions with the largest measuring 63 × 63 × 86 mm in segment 7, a splenic focal lesion measuring 50 × 52 mm, and porta hepatis and portacaval lymph nodes measuring 40 × 48 mm. The patient is suspected of having intra-abdominal involvement. The MDT recommended a new baseline CT CAP and colonoscopy at GRI-E, followed by a medical oncology consult with review of the patient's drug history, and then referral to REMDT.		
تحريراً في ,		
	التاريخ :	23-09-2026
	الوقت :	
	الطبيب المعالج :	
يعتمد ,		



Pathology Report

Name: سميرة محمد حمادين شافعي Age: 64 years.

Clinical data: Left breast mass.

Specimen type: Tru cut biopsy.

Date (Received): 20 /4/2021

Date (reported): 19/5/2021

Lab. Number: 795 /21

Hospital Number: صحة المرأة

Gross Description:

Five greyish white tissue cores, 1.3cm each. totally submitted.

Microscopic picture:

Histopathological examination of sections prepared from the specimen received reveals an infiltrating malignant epithelial growth. The tumor is made up of groups and small sheets of pleomorphic hyperchromatic malignant ductal epithelial cells. Stromal desmoplasia is moderate. Cellular and nuclear atypia are moderate [grade-II].

ER: Strong nuclear staining in 60% of tumor cells.

PR: Strong nuclear staining in 60% of tumor cells.

Her2: No membranous reactivity.

Diagnosis:

Left breast mass. Tru cut biopsy.

Invasive duct carcinoma grade II.

ER: positive, score (8/8)

PR: Positive, score (8/8)

Her2: Negative, score (0)

Prof. Dr. Lubna Omar
Dr. Enas Mohammed Ali



Pathology Report

Name: سميره محمد حساتين شافعي

Age: 64 years.

Clinical data: Left breast mass.

Specimen type: Tru cut biopsy.

Date (Received): 12/4/2021

Date(reported): 18/4/2021

Lab. Number: 795 /21

Hospital Number: صحة المرأة

Gross Description:

Five greyish white tissue cores, 1.3cm each. totally submitted.

Microscopic picture:

Histopathological examination of sections prepared from the specimen received reveals an infiltrating malignant epithelial growth. The tumour is made up of groups and small sheets of pleomorphic hyperchromatic malignant ductal epithelial cells. Stromal desmoplasia is moderate. Cellular and nuclear atypia are moderate [grade II].

Diagnosis:

Left breast mass. Tru cut biopsy.

Invasive duct carcinoma grade II.

For marker study (ER, PR, Her-2, Ki67).

Prof. Dr. Lubna Omar
Dr. Abdulrahman Ibrahim



Women's Health Outreach Program

Name: Samira hasaneen Shafiy

Date: 11/4/2021

Bilateral Mammography & Bilateral High Resolution Ultrasound

Clinical summary:

64yrs female presented by palpable left breast mass since two year

Detailed Findings

Left breast

- ACR a fatty breast parenchyma,
- UOQ spiculated irregular dense mass lesion.
- By ultrasound UOQ 1-3 O'clock hypoechoic irregular spiculated solid lesion with architectural distortion and internal vascularity by Color Doppler, it measures about 3.4x2.4cm
- Thick overlying skin, but normal areola nipple complex.
- Ipsilateral pathological axillary tail LN with focal cortical thickening 7mm and effaced hilum.

Right breast

- ACR a
- No masses or areas of distortion.
- Normal skin/nipple/areolar complex.
- Preserved axillary LNs .

Conclusion:

-Left breast: UOQ malignant looking lesion with ipsilateral pathological LN (BIRADS 5)

-Right breast: (BIRADS 1)

Recommendation: Biopsy and histopathological correlation

Best regards,

Eman Ali

Patient Name: Sameera Mohamed Hasanain Shafie

Date : 25/09/2025

Exam : CT CAP(CT Chest Abd. Pelvis)

Age : 68 Year(s)

Medical ID: 2107/2021

Pat. ID: 18463

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -**** Known case of breast cancer. Compared to last study dated 20/4/2025****Chest:**

- Progressive course of left breast mass lesion measuring about 3.8x 2.4 cm become multiple masses 1.8 x1.3 cm at inner quadrant and the largest at outer quadrant 3.3 x2.6 cm with speculated margins.
- Unchanged enlarged left axillary lymph nodes are seen, being subcentimetric.
- No sizable pulmonary masses, areas of consolidation or cavitary lesions.
- Multiple enlarged mediastinal lymph nodes , the largest measures 20 x 9 mm .
- Right hilar L. N measures 14 x 8 mm.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

Abdomen and pelvis:

- Enlarged liver with apparent size progression of the hypodense focal lesions with the largest one at right lobe segment VII measures 5 x4.1 cm compared to last study segment V measures 1.5 X 1.4 cm
- No intra or extra hepatic biliary dilatation.
- Progression of nodular thickening of both suprarenal glands more evident on the left side measures 2.3 x1.4 cm compared to 1.6 X 2 cm .
- Newly noted subhepatic , and left lumbar peritoneal nodules the largest measures 12 x 10 mm.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Left internal iliac L. N 10 8 mm.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, and I.V.C.
- Under filling of the urinary bladder with no filling defects.
- Bulky uterus showing irregular contour with few myometrial lesions, likely fibroids, for US correlation.
- No abdominal or pelvic masses or collections.
- Right gluteus medius intramuscular fat containing lesion mostly lipoma.
- Still noted sclerosis with increased in number of the widespread osseous formerly lytic lesions for bone scan.
- Wedging of D6, D7 and L1 vertebral bodies.
- L5/S1 spondylitis.

OPINION:***Overall progressive course as the above described changes******Dr. Reham Khedr,, M.D.***

Name : Samira Mohammed Hassanain Shafai
Branch : Dokki
Ref.Physician Prof : Gostaf Roseh Hospital

Date : 25-10-2025 01:35
Medical No. : 233397
Ins.Company : تعاقد مستشفيات

Tc^{99m} MDP SKELETAL SCINTIGRAPHY

History:

- 68-year-old lady, with history of metastatic breast cancer, referred for follow up.

Technique:

- Anterior and posterior whole-body scans were acquired 2 hours post injection of the tracer.

Findings:

- Compared to the previous bone scan dated 04/11/2024, the current study showing widespread skeletal metastases seen at the skull, sternum, both shoulders, scapulae, humeri, spine, bilateral ribs, pelvic bones, femora and right tibia.

Impression:

- Progressive course of the metastatic osseous lesions.

Much oblige
Dr. Alaa Nagi, MD
Nuclear Medicine



Patient Name: Sameera Mohamed Hasanain Shafie	Date : 15/10/2025
Exam : ultrasound ABDOMEN& PELVIS	Age : 68 Year(s)
Medical ID:	Pat. ID: 18463

• **ABDOMINO-PELVIC ULTRASOUND EXAMINATION REVEALED:**

• **Liver: -**

- *Enlarged in size (20cm at Rt MCL) heterogenous parenchymal echo pattern with multiple discrete & confluent hypoechoic focal lesions, the largest is seen at segment VII measuring 8x10cm*
- *No intra hepatic biliary radicle dilatation.*
- *Portal vein is patent but attenuated.*

• **Gall Bladder: -**

- *Average distention and capacity with no stones or mud*
- *. The wall is not thickened.*
- *CBD is not dilated.*

• **Spleen: -**

Average size, homogenous.

• **Kidneys: -**

- *Both are average in size, shape and echogenicity*
- *No stones or dilated pelvicalyceal system*
- *No gross lesion*

Para-aortic and pancreatic area: *show no gross abnormalities.*

No ascites

Urinary Bladder: -

- *Not distended enough however no stones or diverticular out pouching*

Uterus and adnexal: *no abnormalities could be detected*

Opinion:

Hepatomegaly with bilobar focal lesions (metastases)

Colonic gaseous distention

Signature Dr . Nermien Ramadan

PM 1:59 2025/10/15